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Guide to Harm Reduction in the Emergency Department

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Background

CA Bridge aims to make treatment for addiction the standard of care in hospitals and emergency departments (EDs) throughout California. We are most widely known for our model of low-barrier medication for addiction treatment (MAT) that involves starting patients on buprenorphine in the ED and connecting them to ongoing outpatient care. This is a critical starting place for hospitals, but it is not the end. Healthcare workers have an important role in decreasing the negative effects of substance use disorder (SUD) on individuals and their communities.¹

The illicit drug market is unregulated and unstable. Prohibition-based policies, including those criminalizing the sale of illicit drugs, make the illicit drug supply even more dangerous, due to increased potency and added fillers that can be toxic.² While many medical professionals promote an abstinence-based approach to discussing drug use, offering tools and education around safer drug use supports patient health if they continue to use drugs. **Harm reduction is the moral foundation and root practice of offering these tools and services.**

→ This toolkit is designed to assist hospitals in establishing harm reduction programs.

What is harm reduction?

Harm reduction is a set of strategies, practices, and tools developed by and for people who use drugs to reduce the risks associated with drug use. Harm reduction is a legal, pragmatic, and effective approach to assisting people who use drugs, regardless of whether they are using experimentally, regularly, chaotically, or are abstinent.

Harm reduction principles have long been used in healthcare to manage the risks associated with many medical conditions, including alcohol use, diabetes, eating disorders, etc. Counseling is provided on how people can make healthier choices, even if they are still engaging in behaviors that carry risks. **For patients with SUD, harm reduction applies these principles to drug use to reduce the risk of infectious disease transmission, skin and soft tissue infections, and overdose.**

Why should healthcare providers be involved?

While it is common for healthcare workers to encourage abstinence, the data show that **telling people to stop drug use is not effective**.³ Recovery from addiction often requires multiple quit attempts.⁴ When people experience anticipated periods of returning to drug use, healthcare providers have the ability to significantly reduce their patients' health risks through the provision of naloxone, safer drug use supplies, and education.^{4,5,6} Numerous peer-reviewed studies have found that these practices are one of our most effective tools in the fight against drug-related injury and overdose.^{8,9}

"As a physician, the first time I handed one of my emergency room patients using injection drugs a kit with sterile needles and a sharps container, it felt like I was violating a sacred taboo. I looked over my shoulder, fearing some unseen authority would judge me for condoning drug use.

The current overdose crisis demands that doctors shed this type of apprehension. Harm reduction staff and volunteers working outside the traditional medical system provide pragmatic and compassionate assistance that saves lives. They should not be working alone. Providing sterile equipment, naloxone and education on using drugs safely should be the expected practice in every general medical clinic, every emergency department, and every hospital in America."

– Andrew Herring, MD, Co-Founder, CA Bridge

The harm reduction approach establishes the ED as a welcoming, non-stigmatizing health care setting, where people who use drugs can receive resources that significantly improve their quality of life and reduce their risk of disease and death.¹⁰ This therapeutic alliance immediately improves health outcomes and sets the stage for improved health over the long term as patients develop trust and become more receptive to engaging in medical care and addiction treatment.

“Harm reduction is critical to keeping people who use drugs alive and as healthy as possible, and is a key pillar in the multi-faceted Health and Human Services’ Overdose Prevention Strategy.”

— US Department of Health and Human Services - SAMHSA

Why should the ED be involved?

An estimated 11% of adult ED patients screen positive for SUD, putting them at risk for the harms associated with drug use, including overdose, drug-related injuries, and life-threatening infections.¹¹ For many of those at highest risk, the ED is the only place they access healthcare, often after a nonfatal overdose.¹²

The ED is an essential place for people who use drugs to access high-quality care. Research has demonstrated the effectiveness of starting patients on medication for addiction treatment in the ED.¹³ Regardless of a patients’ treatment status, providing harm reduction tools and education can help prevent chronic diseases, such as human immunodeficiency virus (HIV) and hepatitis C (HCV), and reduce the risk of death by overdose.^{14,15,16,17,18} Providing this care in the ED can be a bridge to external harm reduction partners, like syringe service programs (SSPs), and support future attempts at substance use treatment.¹⁹

“Harm reduction is a public health-based strategy to reduce the negative consequences associated with a particular disease or behavior for individuals and their communities. Although it is most often associated with drug use, it also applies to clinicians’ attempts to manage conditions such as diabetes and hypertension, with the goal of encouraging individuals to be as healthy as possible by meeting patients where they are — even if they don’t follow the optimal treatment plan — through teaching the skills necessary to maximize quality of life and minimize morbidity.”

— Strayer et al¹

The ED Harm Reduction Toolbox

Incorporating harm reduction into the ED setting is not a one-size-fits-all process. Some of these strategies may be easily implemented in your ED, allowing for fast uptake; others may be challenging.

Starting with an easy win, such as patient education, has given many hospitals the momentum to push for more complex interventions, such as sterile syringe distribution. This section provides an overview of several harm reduction strategies organized by how easily they can be incorporated into the CA Bridge model, starting with the easiest and ending in the most challenging.

1. [Educate patients](#)
2. [Offer referrals](#)
3. [Distribute naloxone](#)
4. [Display signage](#)
5. [Distribute fentanyl test strips](#)
6. [Distribute safer use kits](#)

Educate patients

Providing education on how to use drugs more safely can reduce patients' risk and improve their health. Navigators play the primary role in patient education, but nurses, clinicians, and ED technicians should also be trained to provide harm reduction materials. While distributing written materials is great for provider education and patient reference, having direct conversations with patients about harm reduction is essential for behavior change. During regular hours when navigators are on duty, they will be the most frequent point of contact for patients. However, other ED providers, especially nurses and clinicians, should also be trained to engage in non-judgmental, non-coercive conversations focused on reducing patients' drug-related risks. These conversations should help patients utilize strategies to reduce the most imminent risks to their health. Incorporating information from the resources below can help ED providers offer high-quality harm reduction education.

Universal precaution messaging

There are some practices that apply to all drug use, no matter what the means of consumption. These **universal precaution messages** are community-driven practices that can help reduce the risk of injury and death related to substance use. These messages can be shared with patients that use drugs regardless of a provider's ability to offer physical harm reduction materials.

- **Expect fentanyl**
 - Fentanyl has been found in many street drugs, including heroin, meth, and cocaine/crack. Before using any street drug, consider that it may contain a fatal dose of fentanyl, and plan accordingly.
- **Never use alone**
 - You can't reverse your own overdose with naloxone. Use drugs with others or ask someone to check on you. Phone services like [Never Use Alone](#) are also helpful, as they take down your address and are prepared to call 911 if you are no longer responsive.
- **Carry naloxone and know how to use it**
 - People who use drugs are the most likely to witness an overdose. You can keep yourself and your community safer by having naloxone available and in the open when drugs are being used. Check out our resource on [How To Use Naloxone To Reverse An Overdose](#) to learn more.

- **Test your drugs**
 - Fentanyl test strips (FTS) are a quick way to find out what else is in your substances, though they are not foolproof. Learn how to [Prevent Overdose with Fentanyl Test Strips](#). Follow the instructions carefully.
 - Another option is to do a ‘test dose,’ which is to consume an extremely small amount of a drug to feel its strength and effect before using more.
- **Start low and go slow**
 - You can always use more, but you can’t ‘un-use’ a drug after you take it. Start with a little bit and use more as you understand the potency better. If you haven’t used drugs in a while, your tolerance may have gone down.
- **Manage your relationship with drugs**
 - Relationships with drugs shift over time. You can maintain a safer relationship with substances by taking control of how you use them, as opposed to just following cravings or external pressures. This could include abstaining from some drugs, taking a break from using, or managing the frequency of your use.

The following resources are available to help you provide patient education:

- [National Harm Reduction Coalition Online Training Center](#): There is a three-course ‘Harm Reduction 101’ curriculum (Foundations of Harm Reduction, Overdose Prevention and Response, and Engaging People Who Use Drugs). This is a free resource for people living in California.
- [CA Bridge Navigation Toolkit](#): Section 3.4, Engaging and Motivating Patients, outlines the practice of motivational interviewing, a conversational style that promotes reflection and behavior change. Using motivational interviewing as a method of patient education can help support them in shifting their behaviors to improve their health.

Drug use specific resources

In addition to universal precautions, there are a variety of resources created for people who use drugs to learn more about reducing harm from drug use. While it can be helpful to give written materials to patients, we strongly recommend that providers, nurses, and navigators read and incorporate these messages into bedside and discharge conversations.

- [Getting Off Right](#): Often referred to as the bible of safe injection, this resource is a text of everything there is to know about safer drug use.
- [Skin Care Guide for People Who Inject Drugs](#): This flyer describes how to care for your skin as a person who injects drugs, as well as signs of infection or injury.
- [Transitioning Routes of Administration: From Snorting to Injecting to Eating to Smoking to Booty Bumping](#): This guide provides advice on transitioning methods of use, including determining the best method for you and comparing methods.
- [Safer Consumption Tips for New Users](#): A series of basic harm reduction tips for people who use drugs.
- [Getting Started with Safe Injection](#): Introduction to injecting drugs, including how to prepare your space and mindset, injection techniques, and aftercare.
- [Rectal Administration: Aka Booty Bumping, Boofing, or Plugging](#): Explanation of how to take drugs via the rectum, including why people do it.
- [Hot Rails](#): Introduction to hot railing drugs (a mix of snorting and smoking), including the necessary materials, techniques, and helpful tips.

Offer referrals

It is important for providers and navigators to connect with local harm reduction programs that can offer harm reduction services and supplies to patients on a long-term basis. Building a connection with local harm reduction programs and other partners can create a robust, multidirectional referral system. This ensures there is ‘no wrong door’ for patients to get connected with a variety of services, including accessing sterile syringes, substance use treatment, and medical care for a drug-related health problem.

Refer to the National Harm Reduction Coalition's [map of California SSPs](#) to find a program near you. Reach out to the program and meet with a staff member to learn how you can work together to support your patients. You can also hand out cards or flyers with the program's information to patients looking for harm reduction resources.

“Syringe sharing is the most common cause of hepatitis C transmission and the second most common cause of HIV and hepatitis B transmission in the United States. These preventable infections are devastating to individuals and families and put cost burdens on taxpayer-supported healthcare and health insurance. For example, the US Centers for Disease Control [reports](#) that the lifetime cost to treat a single case of HIV is \$380,000 in 2010 dollars and the cost to cure a patient of hepatitis C is approximately \$64,500.”

—Drug Policy Alliance, [Syringe Access Bill in California Signed into Law by Governor Newsom](#)

Distribute naloxone

Naloxone distribution is an important first step in providing harm reduction services. Naloxone is an opioid antagonist (blocker) used to reverse potentially fatal opioid overdoses.²⁰ Providing people who are at risk for overdose, and their friends and families, with naloxone to keep on hand and training them to prevent, recognize, and reverse an overdose is one of the most effective ways to prevent overdose deaths.

The California Department of Health Care Services has established the [Naloxone Distribution Program](#) (NDP) to facilitate access to free naloxone. Many entities, such as hospitals, clinics, shelters, libraries, schools, and community organizations, now receive free naloxone and distribute it to the public without a prescription under a standing order. Because EDs have contact with a very high-risk population, it is vitally important that EDs participate in the NDP. Serving an access point for naloxone distribution is becoming standard practice across California, with over 136 EDs participating as of June 2022.

For step-by-step guidance on naloxone distribution in the ED, see the [Guide to Naloxone Distribution](#).



Display signage

Signs offering treatment or asking if people want naloxone, harm reduction supplies, or help with their substance use invites patients to speak openly with health care staff and self-identify drug use or having an SUD. Post signs at registration, triage, hallways, bathrooms, and any place in the hospital that patients visit. Download and print a customizable [Patient-Facing Flyer](#) to put up around your ED. Some hospitals have also created signs specifically focused on harm reduction services.

Distribute fentanyl test strips

Fentanyl and fentanyl analogs are synthetic opioids that are 50 to 100 times stronger than heroin.²¹ Unknowingly consuming heroin, cocaine/crack, meth, MDMA/ecstasy/molly, counterfeit pills, or other drugs mixed or cut with fentanyl is a significant risk for people who use drugs.²² Testing drugs for the presence of fentanyl using FTS is one approach to decreasing this risk.²³ However, it is important to understand that it is becoming increasingly common to use fentanyl intentionally. For those who are intentionally using fentanyl, other harm reduction strategies will be better suited for safer consumption.

Benefits: FTS are inexpensive, disposable testing strips that can detect the presence of fentanyl in a sample. They were designed for urine drug testing but are widely used for checking drugs for the presence of fentanyl.²³ The technique involves dissolving a small amount of powdered drug in water and dipping a FTS in the solution for 15 seconds. These test strips are very sensitive and have proven effective in detecting the presence of fentanyl in a diluted sample.²⁴ While these strips do not provide information on the amount of fentanyl in a sample, they are a useful tool to reduce the risk of overdose.

Limitations: Although FTS can detect fentanyl and most known fentanyl analogs, it is not 100% reliable. False negatives can result because fentanyl is not always mixed evenly throughout a drug. For example, one edge of a pressed tablet may contain no fentanyl, while the other edge may contain a fatal dose. False positives can result when methamphetamine or MDMA are present. Thus, while FTS are an important harm reduction tool, they are not foolproof and should never be relied upon as a sole overdose prevention strategy. FTS distribution should always be paired with universal precaution messaging: carry naloxone, go slow, take turns, and avoid using alone.

The following resources provide more information on fentanyl test strip use and overdose prevention tips:

- [Prevent Overdose with Fentanyl Test Strips](#)
- BTNX (FTS manufacturer): [Harm Reduction with Fentanyl Test Strips](#)
- DanceSafe: [Fentanyl Test Strip Instructions](#)
- National Harm Reduction Coalition: [Fentanyl Use Overdose Prevention Tips](#)

Distribute safer use kits

Safer use kits contain materials people can use to prevent drug-related health issues, such as contracting HIV and HCV, while continuing to use drugs.^{14,15,16,17} These can also promote the self-efficacy required to change people's relationship to drug use. While it may seem taboo because of the stigma of drug use, offering these supplies demonstrates a commitment to health without judgment related to drug use.²⁵ This pragmatic approach of meeting patients 'where they are at' with frank discussions and practical harm reduction resources promotes a therapeutic rapport centered around the patient's health and safety. This rapport opens the door for further conversation about the harm reduction benefits of medications, such as buprenorphine.

California state allows SSPs, physicians, and pharmacists to legally distribute supplies for harm reduction purposes.²⁶ This includes both the possession and distribution of syringes supplied for harm reduction purposes, as well as other materials used for drug use, including safer injection materials, safer smoking materials, naloxone, and others.

"More than 200 studies from around the world have consistently shown improved syringe access reduces the extent of needle sharing and disease transmission - all without increasing the rates of drug use, drug injection or crime."

—California Assemblymember Phil Tang, author of [AB 2077](#)

Supplies can be offered in tandem with naloxone and buprenorphine to provide a full range of options for improving health and safety. Kits should be specific to the type of drug(s) that the patient uses as well as their preferred mode of administration. Examples of safer use kits are listed below.

Injection kits reduce the risk of skin and soft tissue infections, HCV, and HIV due to injection with contaminated needles.^{17,19,27} Contents include items such as sterile syringes and water vials, cooker basins, alcohol prep pads, elastic ties, and sharps containers. Check out British Columbia Centre for Disease Control's guide on [Injection Supplies](#) to learn more.

Smoke kits reduce the transmission of infectious diseases, such as HCV and COVID-19.^{28,29} Additionally, smoking or snorting drugs carries significantly less risk of disease transmission, injury, and overdose than injecting drugs. Anyone who is injecting drugs and is unwilling or unable to stop drug use should be encouraged to switch to smoking or snorting as a harm reduction intervention. Contents include items such as pipes and smoking foil, a mouthpiece to protect lips from burning and decrease disease transmission when sharing pipes, alcohol preps, and copper mesh to prevent inhalation of hot particles. Check out Harm Reduction Action Center's [Safer Smoking Kit](#) guide to learn more.

Snort kits prevent HCV transmission through straw sharing and intranasal abrasions.³⁰ Contents of these kits include straws, fentanyl test strips, plastic razor blades to chop the drugs into a fine powder, and plastic cards to use as a clean surface for preparing drugs for snorting. Check out Harm Reduction Action Center's [Safer Snorting Kit](#) guide to learn more.

"Many drugs that are commonly injected – including heroin, fentanyl, and methamphetamine – may also be smoked or snorted, which is a significantly less risky mode of administration for people who are unwilling or unable to stop using drugs."

—[California Department of Public Health \(CDPH\) Issue Brief: Smoking Supplies for Harm Reduction](#)

Making it Happen at Your Hospital

CA Bridge has worked with dozens of hospitals to incorporate harm reduction principles into their practice. We offer the following steps as a starting point for creating a successful hospital-based harm reduction program. If you would like assistance, contact us at info@cabridge.org. We are here to help.

Create a team

Like any practice-change initiative, launching a new harm reduction program will require leadership from a core group of people. Often, this group includes the navigator and a clinician (or clinical champion) who is most engaged in providing treatment to patients who use drugs. Including passionate nurses, pharmacists, physician assistants, and administrators ensures multidisciplinary buy-in. Start with a regular meeting time and begin creating your plan.

Connect with local harm reduction programs

Understanding your local community of people who use drugs is essential to offering relevant services. Reach out to harm reduction programs and SSPs in your area to learn more about their work and your patients. A close, mutually supportive relationship between the navigator and SSP staff benefits your patients, and getting harm reduction started at your hospital is a great way to forge a strong connection.

You can find information about your nearest SSP using the [California state SSP locator](#) or [national SSP locator](#). If there is not an SSP in your area, it is even more essential to offer these services. Connect with our partners at the National Harm Reduction Coalition at california@harmreduction.org to learn more about your area. Below are some ideas of what to talk about when you connect with programs.

- **Learn about their services and resources.** What safer drug use supplies do they offer? Do they offer HIV and HCV testing? Do they have on-site health professionals, such as a nurse or counselor? What are their hours and locations?
- **Learn about your local community of people who use drugs.** What are the most common drugs and consumption methods? What are the demographics of people accessing their services?
- **Connect with the outreach director.** This is a great opportunity to discuss how your hospital and the SSP can work together to support patients. Ask them the best way to refer patients to the SSP, such as distributing flyers or business cards.
- **Educate them on what you can offer.** Share information about your ED's services for people who use drugs, including offering buprenorphine. Explain how they can contact the navigator for assistance and refer patients to the ED or MAT clinics.
- **Learn about satellite sites.** Is your hospital within the SSP's authorized distribution area? If so, your ED may be able to receive supplies from the SSP and function as a satellite site for redistribution. *Note: As a physician or pharmacist-led program, you don't need permission to distribute syringes. It is [legal in California](#) for physicians and pharmacists to distribute safer drug use supplies for harm reduction purposes.*

Select your interventions

There are many options for integrating harm reduction into the hospital setting. **We recommend starting with patient education on universal precautions and a naloxone distribution program**, which can be implemented using the [Guide to Naloxone Distribution](#). Offering patient education can help you educate other staff about harm reduction in tandem with patients who use drugs. Establishing a naloxone distribution program creates a structure for giving patients free materials to support their health and helps familiarize the ED staff with harm reduction

principles. From here, you can fold in other practices, such as distributing FTS and educational materials. Once these elements are well established, consider adding safer drug use supplies, such as syringes, smoking supplies, and snort kits.

Engage leadership

As you expand your hospital's harm reduction work, it is important to engage leadership. While some harm reduction practices offered in the ED can be launched without institutional approval or policy, doing this may lead to fierce internal resistance. Regardless of internal approval, community backlash or negative press coverage for the hospital may harm your program if you do not have the support of leadership. Taking an incremental approach and continuously strategizing about who needs to be engaged will increase your success.

Order harm reduction supplies

- For more information regarding naloxone distribution in the ED, including step-by-step guidance on ordering and distributing free naloxone, see the [Guide to Naloxone Distribution](#).
- A local SSP may be able to provide harm reduction kits and offer guidance on best practices as your program gets started. To obtain contact information for your nearest SSP, use the [California state SSP locator](#) or [national SSP locator](#).
- Once your program is ready to obtain larger quantities of supplies, the site can apply directly to the CDPH to become a state-approved syringe access program. This gives your hospitals access to harm reduction materials supplied directly from the CDPH Office of AIDS SSP Clearinghouse. [Apply here](#).
- The hospital can purchase supplies directly from programs, such as [Points of Distribution](#) or the [North America Syringe Exchange Network \(NASEN\) Buyer's Cub](#). These programs require applications and operate as an intermediary between manufacturers and the programs distributing materials. This is a great option if your program has a budget to support purchasing materials.

Pilot test your intervention

We recommend starting with the core members of your team who will gain experience by engaging in harm reduction discussions and kit distribution to patients and visitors. This phase gives you an opportunity to identify issues, troubleshoot, and improve workflows. This will ensure the team is prepared to answer questions from staff, patients, and community members when the program is ready for expansion. The pilot phase should last as long as it takes to work out the kinks of your program and train additional team members. Typically, this phase will last between three to six months.

Train hospital staff

Training staff early will increase their success and investment in the program so that, eventually, offering harm reduction supplies is part of the ED workflow.

Hold small groups or individual training sessions. Perspective change happens incrementally. When it comes to deep-rooted issues around drug use, it is essential to give staff space to explore new ideas. Begin with volunteers and essential leaders to maintain positive energy for the program. You won't reach everyone at once, but having a smaller group of passionate early adopters will help bring others along.

Inform all ED staff about the program. Send out notices and informational material through email listservs. Present at ED staff meetings, nursing leadership meetings, and huddles as you scale the program. Make yourself and your core team available to answer questions and discuss barriers and challenges, especially for leadership.

Integrate harm reduction education into existing training systems. Include modules on overdose prevention and harm reduction in annual competencies or nursing training. This ensures that new staff learn about the program and creates a system for ongoing education

Implement hospital policies that support pain management for people who use drugs. Clinicians should provide adequate pain management to reduce in-hospital illicit drug use and patients leaving against medical advice (AMA).³¹ If illicit drug use does occur, respond with medical and behavioral health support rather than a criminal justice/security response. For more information on institutionalizing an appropriate response to in-hospital illicit drug use and possession, see this [Substance Use and Possession Policy](#) from Zuckerberg San Francisco General Hospital.

Help staff understand their roles

Typical responsibilities of various staff members to maintain a harm reduction program include:

Navigators assemble safer use kits based on the individual patient needs, provide education and demonstration of kit contents, and label the kits to be stored in the locked medication room until discharge.

Nurses ensure safer use kits are offered during discharge, especially when the navigator is not on site. Nurses should be educated about the rationale for the program, how to give out supplies, and safer drug use practices.

Clinicians/prescribers ensure patients are offered supplies. Clinicians may distribute themselves, or ensure a nurse or navigator does.

Front desk staff who do greeting, intake, and triage play an important role in naloxone distribution. These staff should be engaged in the overdose prevention element of a hospital harm reduction program. Intake and greeting staff trained to provide naloxone can expand your naloxone distribution program's reach. These front-end staff can distribute naloxone to the public without requiring them to register as a patient. They can also distribute contact information and flyers for local SSPs.

Social workers and case managers help patients connect to social services and ongoing primary care. They also distribute harm reduction supplies and information about local harm reduction programs.

Plan to reach different patient populations

Patients who have had an overdose reversal

One effective strategy is to start with people who are survivors of overdose. Surviving an overdose is a significant risk factor for dying from a future overdose.³² Focusing on this high-risk population ensures your efforts are having an impact on your community's overdose rates.

You should provide patients and their friends and family with naloxone. You should also offer FTS, overdose education, and safer use materials, such as safer smoking or safer injection kits.

To make it easier on your team, create post-overdose reversal order sets that include naloxone, fentanyl test strips, overdose education, and safer use materials distribution. If you have questions about what should be included in these order sets, please reach out to the CA Bridge harm reduction team at info@cabridge.org. For more information on streamlining this process, see this [Harm Reduction Supplies Order Set](#) from Montefiore Medical Center.

All people who use drugs in the ED

As your program expands, begin to offer harm reduction education and materials to all patients and visitors that use drugs in the ED with naloxone, fentanyl test strips, overdose education, and safer use materials provided on-demand.

Keep in mind, creating safer drug use order sets that include naloxone, fentanyl test strips, overdose education, and safer use materials distribution can streamline this process.

Inpatients

As your program grows, navigators can extend their support and supplies to the inpatient unit. These resources can be incredibly beneficial and don't have to stay in the ED. There are times people who come to the ED are admitted to the inpatient unit, and they should have access to the same services. One inpatient model includes an ED-based navigator who floats to units as needed to provide education early in the hospitalization process. Here are some strategies to support people who use drugs in your inpatient unit:

- Naloxone in hand and safe consumption kits should be offered to inpatients. These can be prepared several days prior to discharge and given to the patient at the time of discharge.
- Appropriately treat pain to prevent patients leaving AMA and the use of illicit opioids.
- Coach patients on safer injection techniques at each admission, regardless of if safer injection supplies can be provided.
- Aggressively treat nicotine cravings with nicotine replacement therapy. Patients may require multiple patches or the combination of patches and gum. Consider working to allow patients to smoke on the hospital campus.

Get feedback

As you implement new harm reduction services, ask patients about how they are being received. Through patient interviews, surveys, comment boxes, or questions during a patient's stay, you can learn more about how your program is working effectively and opportunities to grow or change. It is important to remember that your services are only effective if patients are able to access and utilize them. Learning about patient barriers and challenges in accessing your hospital's harm reduction services can help you continue to expand your reach.

Stay connected

If you are implementing these services in your ED, you are on the cutting edge of ED-based care for people who use drugs. Continue to stay engaged with CA Bridge staff and other EDs doing this work to learn from and share best practices and barriers that are emerging as we all learn about implementing these types of services in the ED. If you have questions or want to connect with other EDs that offer harm reduction services, reach out to info@cabridge.org.

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